Primary care funding architecture: pragmatic validation package

v1.1.0 - 8 May 2026

Purpose: proceed with stakeholder validation, rapid review, OIA/data requests, targeted empirical checks and pilot design, without attempting a fully calibrated predictive model at this stage.

1. Executive summary

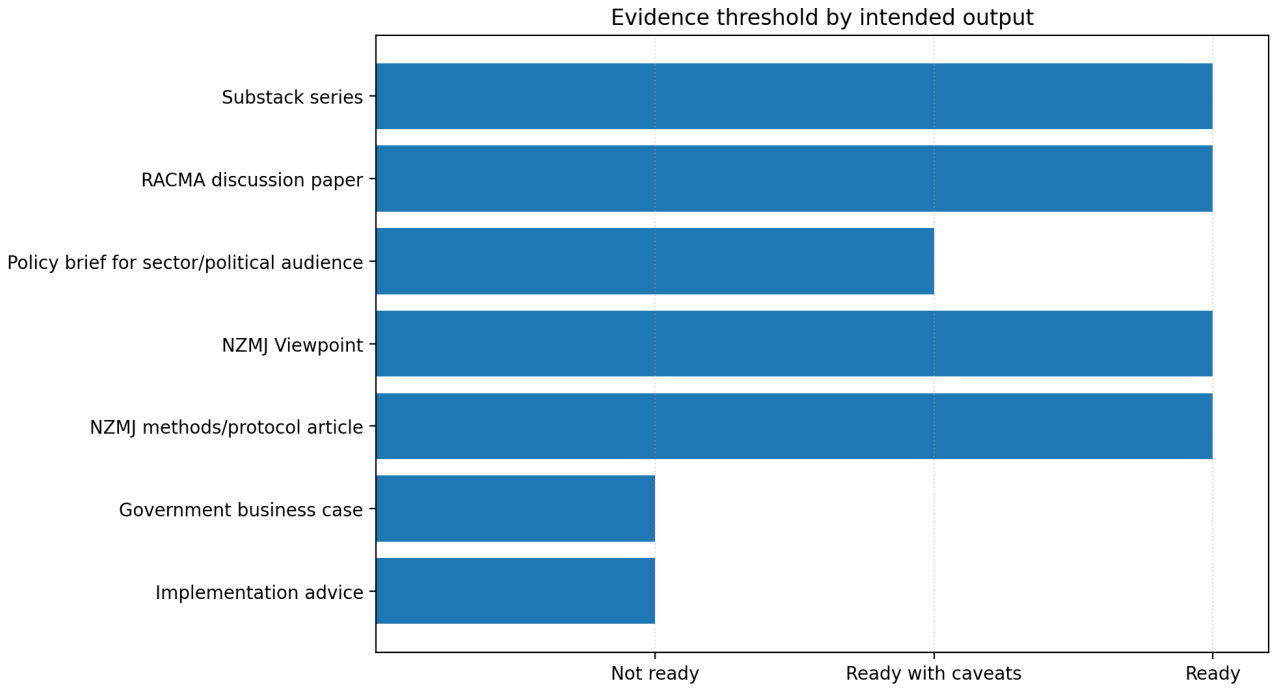
The project should now proceed as a validation and translation programme rather than a full national predictive modelling programme. The current model is source-informed and parameterised, but it remains demonstrative. That is sufficient for policy scoping, RACMA discussion, stakeholder engagement, Substack publication and an NZMJ Viewpoint or methods/protocol article.

A fully calibrated predictive model becomes necessary only if decision-makers require quantified forecasts of ED presentations, hospital admissions, costs, savings, supply effects or implementation business-case outcomes.

The immediate work should therefore focus on validating the games, exposing disagreements, making funding flows observable and testing the five most load-bearing assumptions.

2. Evidence threshold by output

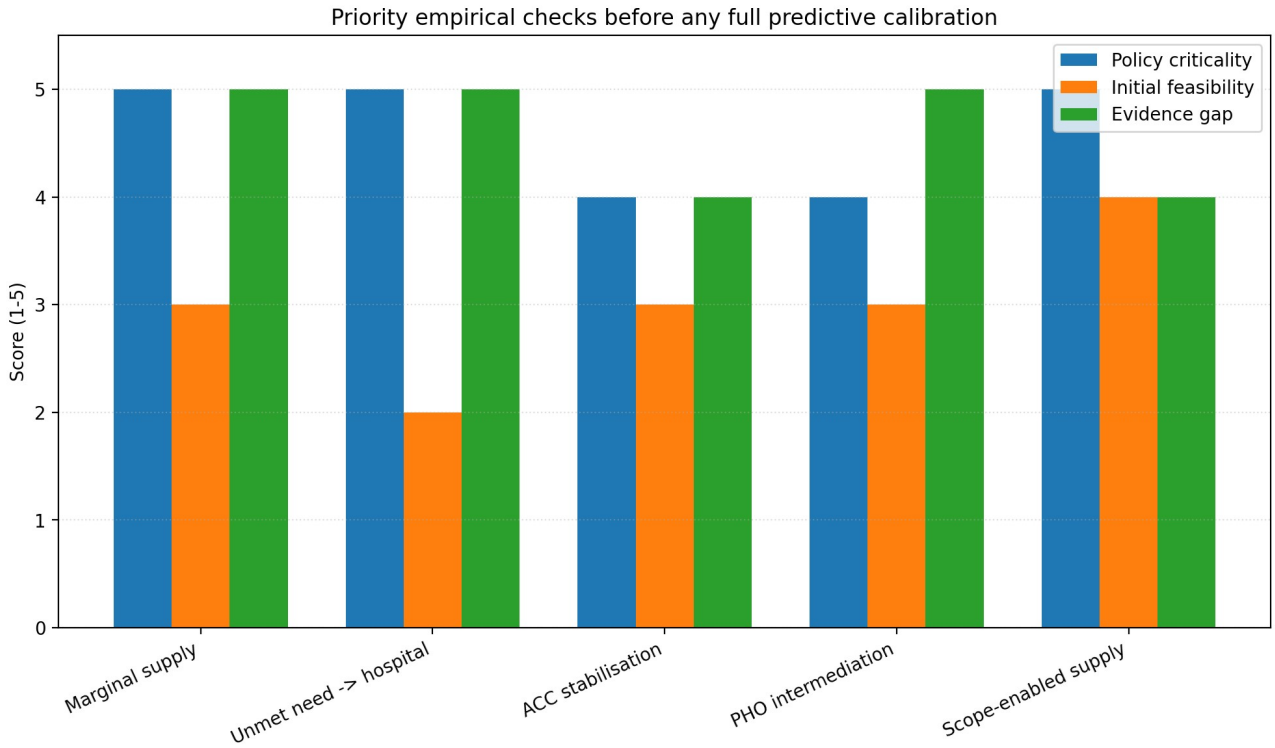
output	minimum_evidence_needed	current_status	do_not_claim
Substack series	Public evidence + transparent theory + caveats	Ready	Quantified forecast, fiscal savings, proven causation
RACMA discussion paper	Game map + audit trail + source-informed demonstrative model + validation plan	Ready	Policy is proven or implementation-ready
Policy brief for sector/political audience	Public evidence + options analysis + model caveats + stakeholder validation plan	Ready with caveats	Guaranteed hospital demand reduction
NZMJ Viewpoint	Conceptual/theoretical argument + policy context + transparent limitations	Ready	Empirical predictive results
NZMJ methods/protocol article	Model structure + audit + standards crosswalk + validation pathway	Ready	Empirically calibrated impact estimates
Government business case	Stakeholder validation + targeted empirical checks + fiscal modelling/pilot results	Not ready	Budget-neutrality or savings
Implementation advice	Pilot/evaluation evidence + operational design + data governance + equity review	Not ready	System can implement without unintended effects



3. Priority empirical checks

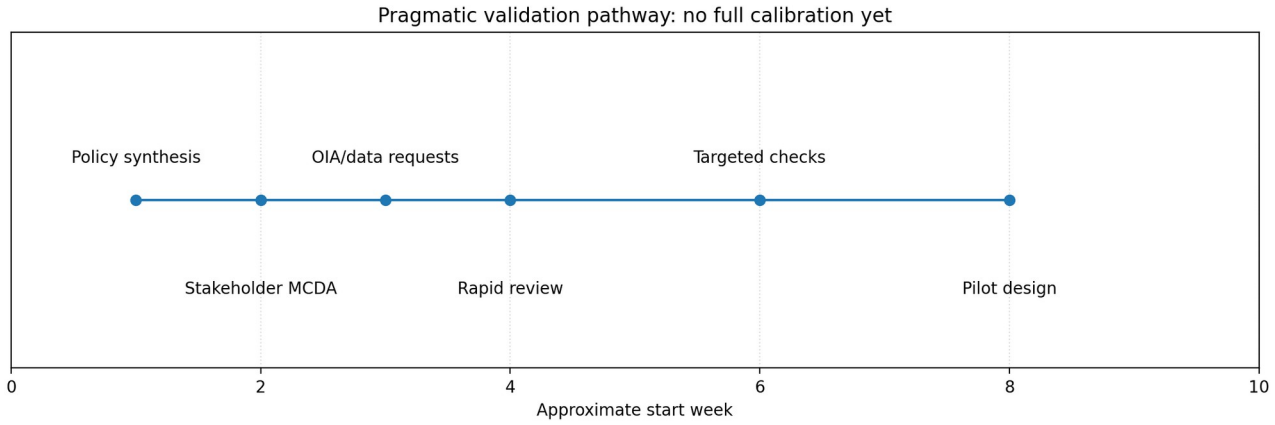
These five assumptions carry the greatest decision weight. They should be tested before a national implementation business case, but they do not need to be fully resolved before public or professional policy discussion begins.

priority_rank	parameter	why_it_matters	proposed_method	decision_use
1	Marginal supply response to contact-based payment	Determines whether a Primary Care Benefits Schedule expands safe upstream activity rather than simply redistributing existing demand.	Practice-level panel analysis; before-after comparison where payment/activity settings vary; stakeholder validation of capacity response.	Whether to proceed with benefits schedule design and which contact types to include first.
2	Unmet primary care to ED/ambulance/hospital conversion	Tests the core wood-from-trees hypothesis that constrained upstream access channels demand into higher-cost hospital pathways.	Linked longitudinal cohort; time-to-event or hazard models linking access delay/unmet need to ED, ambulance, admissions and re-presentations.	Whether upstream funding expansion plausibly deflects hospital demand.
3	ACC activity funding stabilisation effect	Tests whether ACC fee-for-service and treatment payments are mitigating wider primary care supply failure.	Practice-level revenue-mix analysis; compare practices/regions with different ACC revenue shares; sensitivity to ACC payment changes.	Whether ACC treatment-payment policy should be assessed whole-of-system, not in isolation.
4	PHO intermediation transaction-cost / entry-barrier effect	Determines whether PHO reform should focus on function redesign, direct claims, transparency, or abolition/optionalisation.	Mixed-methods analysis: PHO financial-flow audit, onboarding timelines, provider interviews, comparison of PHO pass-through and support functions.	Whether to separate PHO value-adding functions from payment-gateway functions.
5	Scope-enabled provider supply and safety	Tests whether pharmacists, NPs, nurses, allied health and paramedics can safely generate additional contact activity under scope-based benefits.	Pilot evaluation by contact type and provider type; safety audit; re-presentation and escalation analysis; patient experience.	Which contact types can be provider-neutral, which require GP/NP governance, and where patient choice is safe.



4. Pragmatic validation pathway

phase	timeframe	workstream	activities	outputs
1	Weeks 1-2	Policy synthesis	Finalize narrative, limitations, stakeholder-ready artefacts, evidence threshold matrix.	Validation and translation report; one-page brief; Substack synthesis.
2	Weeks 2-4	Stakeholder MCDA preparation	Recruit groups; circulate game map; prepare workshop pack; test scoring instrument.	Workshop pack; scoring workbook; participant briefing.
3	Weeks 3-6	OIA/data requests	Submit OIA requests for capitation formula, PHO advice, NPCD data elements, ambulance and ACC data.	OIA tracker; source update log; request copies.
4	Weeks 4-8	Rapid scoping review	Register protocol internally; run searches; screen studies; extract mechanisms; evidence map.	PRISMA-ScR evidence map; review memo; source matrix.
5	Weeks 6-10	Stakeholder validation	Run workshops/interviews; analyse agreement/disagreement; update game map and MCDA weights.	Stakeholder validation report; revised MCDA.
6	Weeks 8-12	Targeted empirical checks	Assess feasibility of five priority parameters; negotiate data access; prepare pilot protocols.	Data readiness report; empirical check protocols.
7	Months 3-6	Prospective pilot design	Choose pilot contact types and sites; define KPIs; governance; evaluation protocol.	Pilot evaluation plan and implementation-ready measurement framework.



5. Stakeholder validation and MCDA

The highest-value next step is stakeholder validation. The game-informed MCDA should not be treated as proof. It is a deliberative decision-support method that makes assumptions, values, disagreement and risk tolerance explicit.

Suggested stakeholder groups include general practice, nurse practitioners, nurses, pharmacists, physiotherapists, mental health providers, paramedics, rural providers, kaupapa Maori and Pacific providers, PHOs/localities, Health NZ, ACC, ambulance leaders, Treasury/fiscal observers and consumer advocates.

Each game should be scored for reality, harm, hospital-growth contribution, equity relevance, tractability, reform risk and confidence. Policy options should then be scored against access/supply, hospital deflection, equity, rural resilience, fiscal control, gaming risk, market entry, clinical governance, political feasibility and data readiness.

6. OIA and data requests

request_id	agency	request_title	purpose	status
OIA01	Ministry of Health	Current capitation formula, rate tables and implementation model	Make the funding architecture reproducible and auditable.	Draft
OIA02	Ministry of Health	PHO roles, functions and options for change advice H2025067082	Understand unreleased advice on PHO remit/function and options for change.	Draft
OIA03	Health New Zealand	Primary care payment flows, PHO pass-through and provider onboarding	Test PHO intermediation and transaction-cost hypotheses.	Draft
OIA04	Health New Zealand	National Primary Care Dataset metadata and aggregate access data	Prepare stakeholder validation and future empirical checks using NPCD fields.	Draft
OIA05	Health New Zealand / Ambulance Team	Ambulance conveyance, alternative disposition and handover indicators	Test ambulance as upstream access infrastructure and hospital-deflection lever.	Draft
OIA06	ACC	ACC primary care treatment payments by provider type and region	Test whether ACC activity payments stabilise primary care supply.	Draft

The OIA pack in this release contains draft request text for each item. The priority is not to obtain every dataset immediately, but to establish which funding and data flows are observable, which are withheld, and which would require formal research agreements.

7. Rapid scoping review

The rapid review should be PRISMA-ScR aligned and should map mechanisms rather than only outcomes. The key question is: what evidence exists that funding architecture affects access, marginal supply, market entry, equity, ambulance/urgent-care pathways and hospital demand?

The review should produce an evidence map linked to the 14 games and the five priority assumptions.

8. Prospective pilot/evaluation design

pilot_domain	pilot_question	primary_outcomes	design
Benefits schedule contact types	Can defined contact benefits expand safe upstream care?	Encounter volume, 7-day access, same-day urgent access, re-presentation, patient fee burden	Stepped-wedge or matched comparison; prospective process evaluation
Scope-enabled providers	Which provider types safely generate additional activity within scope?	Completed contacts, GP substitution, escalation, safety events, patient experience	Contact-type pilot with clinical governance and safety audit
Ambulance alternatives	Do funded alternatives reduce ED conveyance without unsafe re-presentation?	Non-conveyance, ED presentation within 72h/7d, response times, patient satisfaction	Interrupted time series or controlled before-after
Direct claims / PHO optionality	Do direct claims reduce transaction cost and entry barriers?	Onboarding time, payment lag, administrative cost, claim rejection, patient access	Process evaluation plus comparative implementation study

The pilot pathway should test contact types, provider scope, ambulance alternatives, direct claiming and equity protections before any national-scale implementation. A stepped-wedge or matched comparison design is preferable where feasible.

9. Recommended language

Preferred formulation: The current work is a structured, source-informed and falsifiable policy hypothesis. It is sufficient to open the policy conversation and design validation. It is not yet a calibrated estimate of effect size.

Avoid claiming: quantified ED reduction, fiscal savings, proven PHO causation, or definitive market failure without targeted empirical evidence.

10. Source anchors

SRC01 - Ministry of Health capitation reweighting: https://www.health.govt.nz/strategies-initiatives/programmes-and-initiatives/primary-and-community-health-care/capitation-reweighting

SRC02 - Cabinet paper: Primary Health Care Funding Improvements: https://www.health.govt.nz/system/files/2025-07/Cabinet%20paper%20-%20Primary%20Health%20Care%20Funding%20Improvements%20Redacted.pdf

SRC03 - Health NZ National Primary Care Dataset and health target: https://www.healthnz.govt.nz/about-us/what-we-do/planning-and-performance/primary-care-tactical-action-plan/national-primary-care-dataset-and-new-primary-care-health-target

SRC04 - Ministry of Health PHO finances briefing H2025069314: https://www.health.govt.nz/system/files/2025-11/H2025069314-Briefing-PHO-finances-a-summary-of-available-information.pdf

SRC05 - NZ Health Survey 2024/25 annual update: https://www.health.govt.nz/publications/annual-update-of-key-results-202425-new-zealand-health-survey

SRC06 - Closed books study - PubMed abstract: https://pubmed.ncbi.nlm.nih.gov/38452229/

SRC07 - ACC provider payment page: https://www.acc.co.nz/for-providers/invoicing-us/paying-patient-treatment

SRC08 - Health NZ Ambulance Team: https://www.healthnz.govt.nz/about-us/what-we-do/programmes-and-initiatives/the-ambulance-team

SRC09 - Health NZ emergency ambulance national performance reports: https://www.healthnz.govt.nz/publications/emergency-ambulance-service-national-performance-reports-2025

SRC10 - Ministry of Health OIA requests page: https://www.health.govt.nz/about-us/contact-us/oia-requests

SRC11 - PRISMA-ScR: https://www.prisma-statement.org/scoping

SRC12 - ISPOR MCDA introduction: https://www.ispor.org/heor-resources/good-practices/article/multiple-criteria-decision-analysis-for-health-care-decision-making---an-introduction

SRC13 - STRESS guidelines: https://www.equator-network.org/reporting-guidelines/strengthening-the-reporting-of-empirical-simulation-studies-introducing-the-stress-guidelines/